



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : نيفين جاد سعيد ناروز
المحافظة : القاهرة الحى / المدينة : عابدين
رقم البطاقة : 27603050100101 رقم التليفون : 01287957895
العنوان : القاهرة
العمر (تاريخ الميلاد) : 05-03-1976 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 20-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة :
الاسم : د/ندى حسن الوظيفة :
الاسم : د/نرمين رمضان الوظيفة :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة الأبحاث المرفقة
رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: نيفين جاد سعيد نارو
National ID: 27603050100101



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



النقابة العامة للمحامين
مشروع الرعاية الصحية
للمحامين وأسرهم

شهادة

تشهد النقابة العامة للمحامين بان الأستاذ/نيفين جاد سعيد ناروز

المقيدة برقم (٢٦٤٣٠٠) غير مشتركة بمشروع العلاج بالنقابة
العامة لسنة ٢٠٢٥

وهذه شهادة منا بذلك لتقديمها الى من يهمه الأمر دون الدنى
مسئولية على النقابة.

وتم سداد الرسم بموجب ايصال رقم (١٠٠٠٣٦٧٥٦)

بتاريخ ٢٠٢٥/١١/١٧ رقم قومى (٢٧٦٠٣٠٥٠١٠٠١٠١)

مدير
إدارة الرعاية الصحية
مشروع الرعاية الصحية
مراجعة
التوقيع/ ١٧/١١/٢٥



معامل حسّاب
Hassab Labs

معامل الفا

١6191



المختبر



٢٠٢٤/٠٧

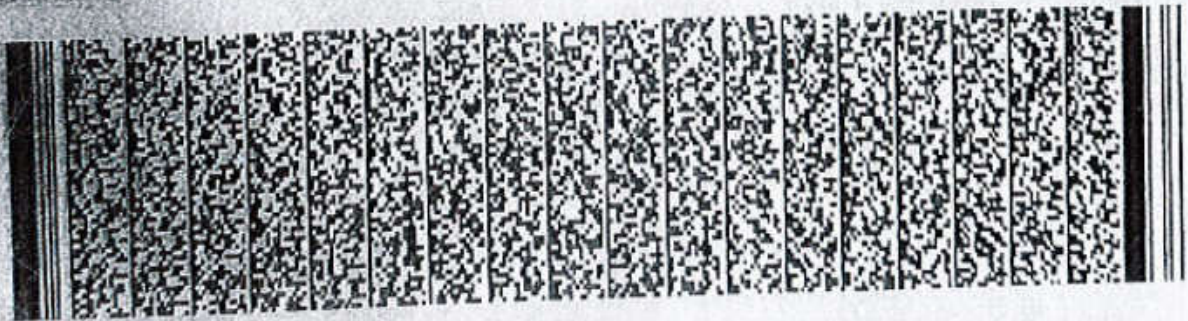
٢٧٦٠٣٠٥٠١٠٠١٠١

محاميه حر



أنثى
مسيحية
متزوجة
مدحت بطرس عزيز عجائبي
البطاقة سارية حتى ٢٠٣١/٠٧/١٥

١



جمهورية مصر العربية
بطاقة تحقيق الشخصية



نيفين

جاد سعيد ناروز

٢٦ ح الساقيين

عابدين - القاهرة

١٩٧٦/٠٧/١٥

٢٧٦٠٣٠٥٠١٠٠١٠١

KQ8832647

جمهورية مصر العربية
 وزارة الداخلية
 بطاقة تعريف شخصية

نيفين
 جاد سعيد تاروز
 ٢٦ ح الشقق
 عابدين - القاهرة

٢٧٦٠٣٠٥٠١٠٠١٠١
 ٢٧٦٠٣٠٥٠١٠٠١٠١

KA8832647

٢٠٢٤/٠٧ (٣٧٦٠٣٠٥٠١٠٠١٠١)
 مصرية
 انثى
 مدينت بورس عراب عابدين
 البطاقة سارية حتى ٢٠٣١/٠٧/١٥

٢٠٢٤/٠٧ (٣٧٦٠٣٠٥٠١٠٠١٠١)
 مصرية
 انثى
 مدينت بورس عراب عابدين
 البطاقة سارية حتى ٢٠٣١/٠٧/١٥

MDT FORM

Name:	Niveen 3ad Saeed .	MRN:	347
Age:	50 Y	ID:	27603050100101
Menopausal Sta	AMMENORRHEA POST NACT	Phone:	01201109076
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	-VE
CO Morbidity:	M:FREE	Reason Of MDT:	POST OP

Clinical Data:

BX: LEFT IDC
ER: 8/8 PR: 7/8 HER2: +3 KI67: 25

Sonomammography:

Mets Workup:

History:

LEFT IDC RECIEVED NACT UNDERWENT LEFT SM+SLNB +EXPANDER INSERTION

Diagnosis:

Left breast cancer (Post-NACT), Left simple mastectomy & Sentinel lymph nodes biopsy:

- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.
- Deep margin: Free.
- Sentinel lymph nodes: Six lymph nodes with no viable metastatic deposits, LNs (0/6)

(Multiple lymph nodes showing effect of therapy).

- Histologic response to therapy: Pathological complete response to therapy.

STAGE:ypT0,ypN0(SLN)

Recommendations:

FOR POSSIBLTY OF ADJUVANT TTT

MDT Outcome & Plan

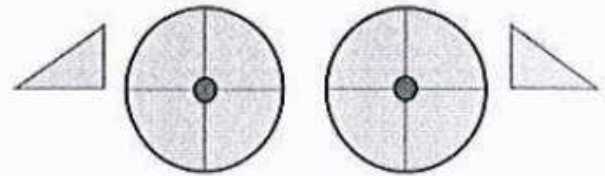
FOR ADJ PHESGD up yo 17 cycles

PORT

ADJ femara consider zoladex after E2 LEVEL if still pre menopausal
zometa every 6 ms

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هاني عورت	د. هاني عورت				

Patient Name : الأستاذة / نيفين جاد سعيد تاورز	Request No. : 13005329
Patient No. : 27603050100101	Request Date : 07/03/2026
Path No. : 631/26	Verification Date : 12/03/2026
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 50 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.

Nature of Specimen: Left simple mastectomy & Sentinel lymph nodes biopsy.

Gross Examination:

- * Left simple mastectomy specimen measured 22x16x5 cm, with covering skin ellipse measured 22x7 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site 11x7x3 cm, located 0.5 cm from the least deep margin.
- * Sentinel lymph nodes: Fibrofatty tissue piece measured 5x5x1 cm, entangling 6 lymph nodes (Only 5 submitted for frozen), the largest measured 3x2x0.5 cm.
- * 17 Blocks:
- 1:13- T.B.S., 14- N&A., 15- Deep margin., 16&17- SLNs..
- Gross was done by; Dr. Heba Hassan.

** Frozen (intra-operative) results:

- Five sentinel lymph nodes free of viable metastatic deposits (Six lymph nodes are detected but only five submitted for frozen).

** Frozen was examined by Prof. Dr. Gamal abd-El-Aty.

Microscopic Examination:

- * Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: Absent.
- * Additional findings: Effect of therapy with microcalcifications.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Sentinel lymph nodes: Six lymph nodes free of viable metastatic deposits (0/6) (Multiple lymph node showing effect of therapy).

Verified by :

Dr.Duaa Mohamed

Printing Date : 25/03/2026 12:28:56

Page 1 of 2

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / نيفين جاد سعيد تاروز	Request No.	: 13005329
Patient No.	: 27603050100101	Request Date	: 07/03/2026
Path No.	: 631/26	Verification Date	: 12/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 50 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

* Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

Left breast cancer (Post-NACT), Left simple mastectomy & Sentinel lymph nodes biopsy:

- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.
- Deep margin: Free.
- Sentinel lymph nodes: Six lymph nodes with no viable metastatic deposits, LNs (0/6) (Multiple lymph nodes showing effect of therapy).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0 (sln).

Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr.Duaa Mohamed

Printing Date : 25/03/2026 12:28:56

Page 2 of 2

Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / نيفين جاد سعيد تاورز	Request No. : 13004506
Patient No. : 27603050100101	Request Date : 25/08/2025
Path No. : 2625/25	Verification Date : 28/08/2025
Sex : Female	Financial Category : جوسفان روسي - نفقة الدولة
Age : 49 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast 12 O'clock lesion (BIRADS 4a) & 1 O'clock lesion (BIRADS 4c).
Nature of Specimen:

- 1- Left breast 12 O'clock lesion, Tru-cut biopsy, collected on 24/08/2025.
- 2- Left breast 1 O'clock lesion, Tru-cut biopsy, collected on 24/08/2025.

Gross Examination:

Two specimens were received as follow:

- 1- Left breast 12 O'clock lesion: Two grey white tissue cores, measured 1 cm & 0.7 cm long. Totally submitted.

- 2- Left breast 1 O'clock lesion: Four grey white tissue cores, the longest was 0.7 cm. Totally submitted.

Technique:

- Sections were prepared from the paraffin block (2), processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:

- 1- Left breast 12 O'clock lesion:

Histopathological examination revealed cores of breast tissue showing diffuse stromal fibrosis and focal adenosis. Some of the proliferating ducts are irregularly dilated. The ducts are lined by inner columnar and outer myoepithelial cells.

- 2- Left breast 1 O'clock lesion:

- * Histological type: Invasive duct carcinoma.
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * 5% Intermediate grade DCIS (Solid pattern).
- * Angioinvasion: Absent.

Verified by :

Dr.Duaa Mohamed



Patient Name : الأستاذ / نيفين جاد سعيد تاورز	Request No. : 13004506
Patient No. : 27603050100101	Request Date : 25/08/2025
Path No. : 2625/25	Verification Date : 28/08/2025
Sex : Female	Financial Category : جوسف روسي - نفقة الدولة
Age : 49 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

- * Perineural invasion: Absent.
- * IPT (Positive control showed appropriate reactivity) (On block-2):
 - ER: Strong nuclear reaction in 70% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 50% of tumor cells (score 7/8).
 - Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
 - Ki67%: 25%.

Diagnosis:

- 1- Left breast 12 O'clock lesion, Tru-cut biopsy:
 - Fibroadenosis.
- 2- Left breast 1 O'clock lesion, Tru-cut biopsy:
 - Invasive duct carcinoma, Grade 2, with 5% Intermediate grade DCIS (Solid pattern).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Positive (score 3+).
- Ki67%: 25%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane

Verified by :

Dr.Duaa Mohamed

Printing Date : 01/09/2025 09:03:51

Page 2 of 3

Printing By : .marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

OP Notes

Patient: **15467218 : Neven Gad Sald**

Female, EGYPTIAN, 49 Year(s), Tel: 93095818

Department: BREAST SURGERY

Doctor : AISHA MASOUD
SLOUM AL SHUKAIRI

Consultant: AISHA MASOUD
SLOUM AL SHUKAIRI

Vital Signs

Examination Notes

23/07/2025 11:07 Aisha Masoud Sloum Al Shukairi - Breast Surgery

Today discussed in MDT , need further workup

CT staging and breast MRI.

Patient called , she is in Salalah . she will come back on 29/7

she is informed about the need for MRI breast

OPD on 5/8 to disclose the diagnosis and CT scan (booked as walk in the same day)...

Diagnosis

Final: Occult breast cancer

Procedures

Date	Procedure	Done Date	Report
23/07/25	CHEST+ABDOMEN+PELVIS CT	05/08/25 11:30:08	Report Date : 05/08/25 11:46:12 CT Abdomen and pelvis with IV contrast (Portovenous) Indication: left breast occult cancer for staging. Comparison: None. Findings: The liver is normal in size and shape. No focal mass lesions seen. Portal vein is patent. There is mild intrahepatic biliary dilatation. The gallbladder showing 3 cm gallstone with no acute complications. Spleen, pancreas and both adrenals are unremarkable. Both kidneys are normal in size, shape and position. No renal or ureteric calculi. No hydronephrosis or hydroureter. Left upper pole Bosniak 1 simple renal cortical cyst measuring 5.3 x 5 cm size. The stomach is distended and appears unremarkable. The small bowel and large bowel are unremarkable. Urinary bladder is grossly unremarkable. No free fluid seen.No evidence of pneumoperitoneum. IUCD seen in the endometrial cavity, unremarkable ovaries, no pelvic mass. No enlarged abdominal lymph nodes are seen.

ID: 4827759

Name: Nevein Gad Saeed Naroz

Date: 20/09/2025

Referred by: Prof. Mohamed Alkilani

Echocardiography Report

Parameter	Value	Normal range for age
AO	2.9	2.0 - 4.0 cm
LA	3.4	2.0 - 4.0 cm
LVED	3.8	3.5 - 5.6 cm.
LVES	2.3	- 3.5 cm
IVS(D)	0.8	0.6 - 1.1 cm
LVPW(D)	0.8	0.6 - 1.1 cm
FS%	41 %	25 - 45 %
EF%	73 %	> 55%
RV(D)	2.0	0.7 - 2.5 cm

Measurements (M-mode):

Comment:

♦ Normal LV internal dimensions and normal global contractility.

♦ No evidence of regional wall motion abnormalities at rest.

Grade I, pattern of impaired LV diastolic function.

Mildly thickened mitral valve (0.65 cm), with good excursion.

Mild mitral incompetence. Regurgitant jet area = 2.1 cm².

♦ Normal aortic valve and normal flow pattern.

Mild tricuspid incompetence. PPASP = 29 mmHg.

No evidence of significant pulmonary hypertension or RV pressure overload pattern.

♦ No detectable intra cardiac masses or thrombi.

♦ Normal pericardium and no detectable pericardial effusion.

Best regards,

Mohamed Shalaby
MD Cardiology
Membership ASE

Name: Nevein Gad Saeed Naroz

ID: 4827759

Date: 18/09/2025

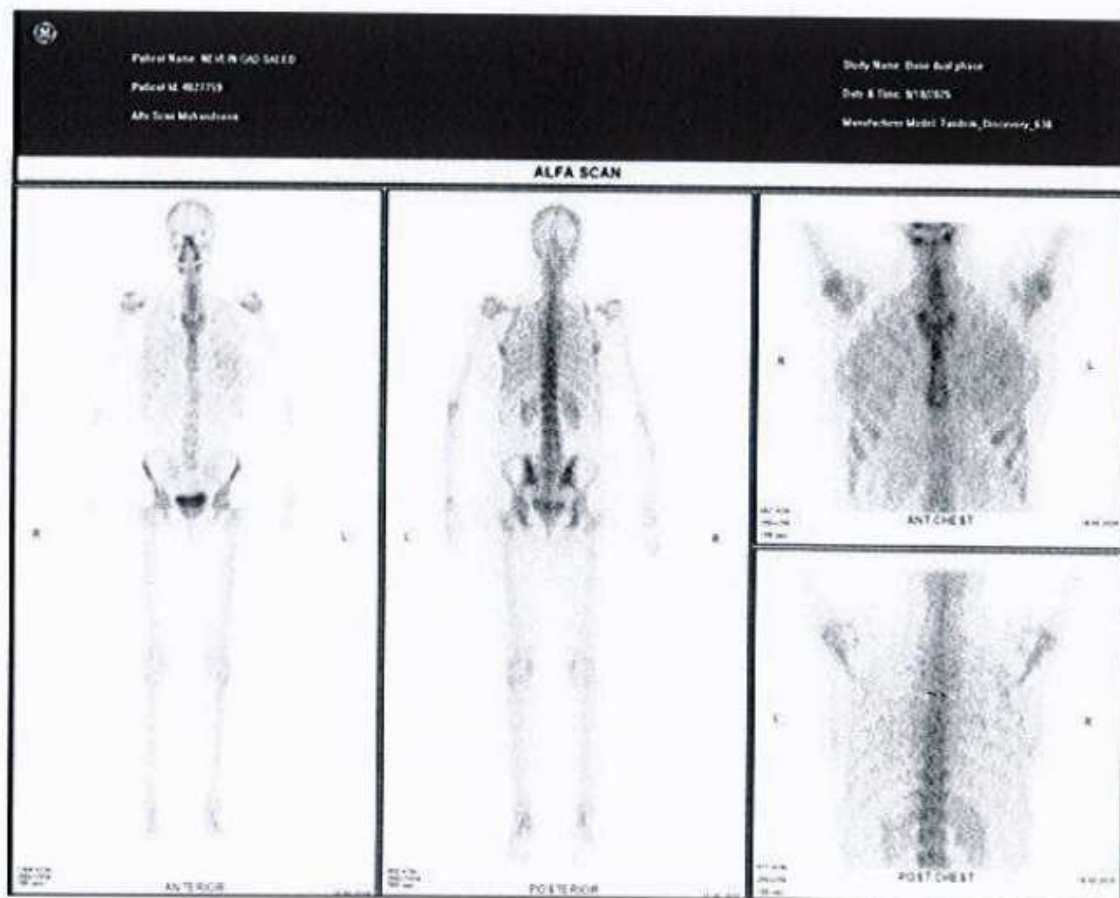
^{99m}Tc -MDP Whole body Bone Scan

Clinical data:

Indication	Left breast cancer.
Tumor markers	Not available.
Skeletal trauma/ surgeries	No
Previous imaging	Not available
Previous bone scan	No

Technique:

Injected dose	740 MBq
Images	Delayed whole body + spot images



Findings:

- No abnormal tracer uptake is seen over the surveyed skeleton that would account for bone metastases.
- Mild degenerative changes of the spine are noted.

Impression:

- No scintigraphic evidence of bone metastases.

Dr. Sherif El-Refaei, MD, EBNM

Dr. Hagar Nageh

OP NotesPatient: **15467218 : Neven Gad Said**

Female, EGYPTIAN, 49 Year(s), Tel: 93095818

Department: BREAST SURGERY

Doctor : AISHA MASOUD
SLOUM AL SHUKAIRIConsultant: AISHA MASOUD
SLOUM AL SHUKAIRI**Vital Signs****Examination Notes****06/08/2025 12:53 Aisha Masoud Sloum Al Shukairi - Breast Surgery**

Telemedicine appointment to trace MDT plan:

MDT Breast:

Case of left breast cancer with axillary LN mets, triple positive

Imaging reviewed: 1 small lesion in the breast, the bigger lesion is benign (hamartoma), pathologic axillary LN

Staging: lung nodules for follow up

Plan:

1. For NAC DCHP x6

2. Bone scan

3. ECHO

4. Surgical plan: BCS + axillary staging. (clip inserted)

5. Genetic testing

patient called, she is not working (her husband is a teacher) , she is willing to buy the medication from Egypt and get the injections and follow up in Royal Hopistal
referral to oncology given for appointment .**Diagnosis****Final:** brease cancer**Referrals****Internal Referral**Referred to Medical Oncology On: 06-08-2025 13:00

Requested By Aisha Masoud Sloum Al Shukairi(Breast Surgery)

Notes: newly diagnosed breast cancer , discussed in MDT for neuadjuvent systemic therapy kindly give appointment

Replied by Hind Mohammed Ibrahim Ahmed

Unit (MEDICAL ONCOLOGY) On: 06-08-2025 13:06

Opinion: Thank for referral:as per Hospital role,expatriate patient to be managed at private sector Dr.Sahar will contact the patient to explain the plan

Appointments

Printed on 11-Aug-2025 10:47 by (86012) Sahar Mohamed Awad

Procedure Visit

Patient: **15467218 : Neven Gad Said**

Female, EGYPTIAN, 49 Year(s), Tel: 93095818

Department: RADIOLOGY

Doctor : AISHA MASOUD
SLOUM AL SHUKAIRI

Consultant: SAQAR HAMED
ISSA AL TAI

Vital Signs

Lab Investigations

Execute Date	Test Name	Released Date
03/08/25 11:13	Histopathology Examination	05/08/25 11:47
GROSS :- Recieved are multiple linear cores of grey soft tissue measuring in aggregate 1.5 x 0.6 cm. All are submitted in one block. SYNOPTIC REPORT :- Left breast lesion, needle core biopsy: ----- Tumor Histologic type: Invasive duct carcinoma, NST. Category B5b. Tumor Grade: - Tubular score: 2. - Nuclear pleomorphism score: 3. - Mitotic rate count: 9/10HPF, score: 2. - Overall provisional grade: 2. DCIS garde, patterns & necrosis: high grade, cribriform with focal necrosis. Microcalcifications: Present, focal. Biomarker studies: -Oestrogen receptors: Positive, 80% of tumour cells show moderate nuclear staining (Allred score 5+2 = 7/8). -Progestrone receptors: Positive, 60% of tumour cells show moderate nuclear staining (Allred score 4+2 = 6/8). -HER-2 immunostain score: -Positive, score 3+ (complete, intense circumferential membrane staining in more than 10% of invasive tumour cells). ----- Snomed Classification : T-04010 - BREAST FEMALE. NOS. M-85003 - Carcinoma-invasive duct US - URGENT SMALL BIOPSY Z - . . . COPY OF THE REPORT SENT TO - "THE CANCER REGISTRY", THE ROYAL HOSPITAL. P24 - 24 workload points Reporting Doctors:- DR.ZAKIYA IBRAHIM HAMED AL AJMI Consultant Department of Histopathology		

Printed on 11-Aug-2025 10:49 by (86012) Sahar Mohamed Awad

PATIENT NAME	NEVIEN SAAD SAED
DATE	01 /02 /2026

BREAST HIGH RESOLUTION ULTRASONOGRAPHY REVEALED:

Indication: Follow up after neoadjuvant chemotherapy of pathologically proved Left breast carcinoma.

- Homogenous glandular background parenchyma.

Left breast:

- Complete disappearance of the previously described malignant mass seen at 2 o'clock position of the left breast.
- The previously described well circumscribed lesion seen in the left breast at 11-12 o'clock position ,zone 1 ,now measures (6 x 2 cm)...mostly fibroadenolipoma.
- No other suspicious masses in other quadrants.
- Multiple cysts are seen scattered in the left breast parenchyma,the largest seen at 1 o'clock and measures (11 x13 mm).
- Normal skin, subcutaneous fat and nipple areola complex on both sides.
- Restoration of the shape and hila of the previously described left suspicious axillary node.

Right breast :

- Homogenous glandular background parenchyma.
- Multiple subcentimetric cysts are seen scattered .
- The previously described well circumscribed lesion seen at 12 o'clock measures (8 x 6 mm).
- Normal skin, subcutaneous fat and nipple areola complex.
- Non specific right axillary LNs.